- Anemia
- Pruritus
- Jaundice
- Hypertension
- Pericarditis
- Peripheral neuropathy, dialysis dementia

Diagnosis

- History collection
- Physical examination
- Identification of reversible renal disease
- Renal ultrasound
- CT scan / Doppler
- Renal biopsy
- Blood-BUN, creatinine, electrolytes, Hb level
- Urine analysis
- Renal biopsy

Management

Medical

- Calcium and phosphorous binders and phosphate binders
- Anti hypertensive drugs-to control BP
- Hypoglycemic agents-to reduce and maintain blood sugar level
- Fluid restriction-600 ml plus previous day output)
- Diuretics-to increase the output
- Erythropoien-to maintain RBC count
- Dialysis
  1. Peritoneal
  2. Hemodialysis



75% of kidney function can be lost before it is noticeable

Diet

- Low protein - 0.8 gm protein/kg/day with normal carbohydrate
- Low potassium (avoid bananas, dry fruits, fruit juices), low phosphorus (soak vegetables 24 hr prior to cooking).
- Fluid allowances per day is 500 ml to 600 ml plus previous day urine output

Surgical management

- Renal transplantation

Nursing management

1. Assessing fluid status and ideentifying potential source of imbalance
2. Implementing a dietery program to ensure proper nutrional intake
3. Promoting positive feelings by encouraging increased self-care and greater independence
4. Provide explanations and information to the patient and family concerning ESRD, treatment options and potential complications.
5. Provide emotional support to the patient and family
6. Health education on diet and fluids.

Complications

- Hyperkalemia
- Pericarditis
- Pericardial effusion
- Pericardial tamponade
- Hypertention
- Anaemia
- Increased incidence of fracture

2.16 Renal Stone/Renal Calculi / Urolithiasis

A stone is a hard, solid mass that can form in the gallbladder, bladder, and kidneys. Kidney stones usually originate in kidneys.

Definition

Urolithiasis refers to the presence of stone (Calculi) in the urinary tract.

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